

AAC Operations Standard

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1. INTRODUCTION

- 1.1 For the context of this document, the term “AACs” will be used collectively to refer to Active Ageing Centres and Active Ageing Centres (Care). The term “CICs” will be used to denote the Centers In-Charge of AACs, which could be the Center Managers or Center Supervisors.
- 1.2 This Standard defines the operations requirements for the purpose of establishing and maintaining operational consistency, service quality and auditability across all AAC teams.
- 1.3 This Standard is applicable to all AACs under NTUC Health and applies to all full-time and part-time staff of AACs who are involved in the provision of AAC services.
- 1.4 This document should be read in conjunction with organisation-wide policies, standards, frameworks, manuals and guidelines, including but not limited to the following:
 - Data Governance Framework
 - Data Protection Act (PDPA) Policy
 - Privacy Policy
 - Data Access Management Policy
 - Data Retention Policy
 - Data Classification Policy
 - Procurement Policy
 - Personal Data Protection Act Incident Response Framework
 - Service Standards Manual
 - House Style Guidelines for staff use when drafting official letters, emails and other documents
 - Step-by-step guide to file encryption
 - AAC Playbooks
- 1.5 In addition to internal documents, AACs are expected to comply with all service and reporting requirements from funders and take reference from the following documents:
 - Service Requirements for Active Ageing Centre Service
 - Operations Guide for Active Ageing Centre Service
- 1.6 In this standard, the terms “should”, “shall” and “must” refer to requirements that are expected to be adopted and complied, and where deviations need to be justified and discussed at the AAC management level.

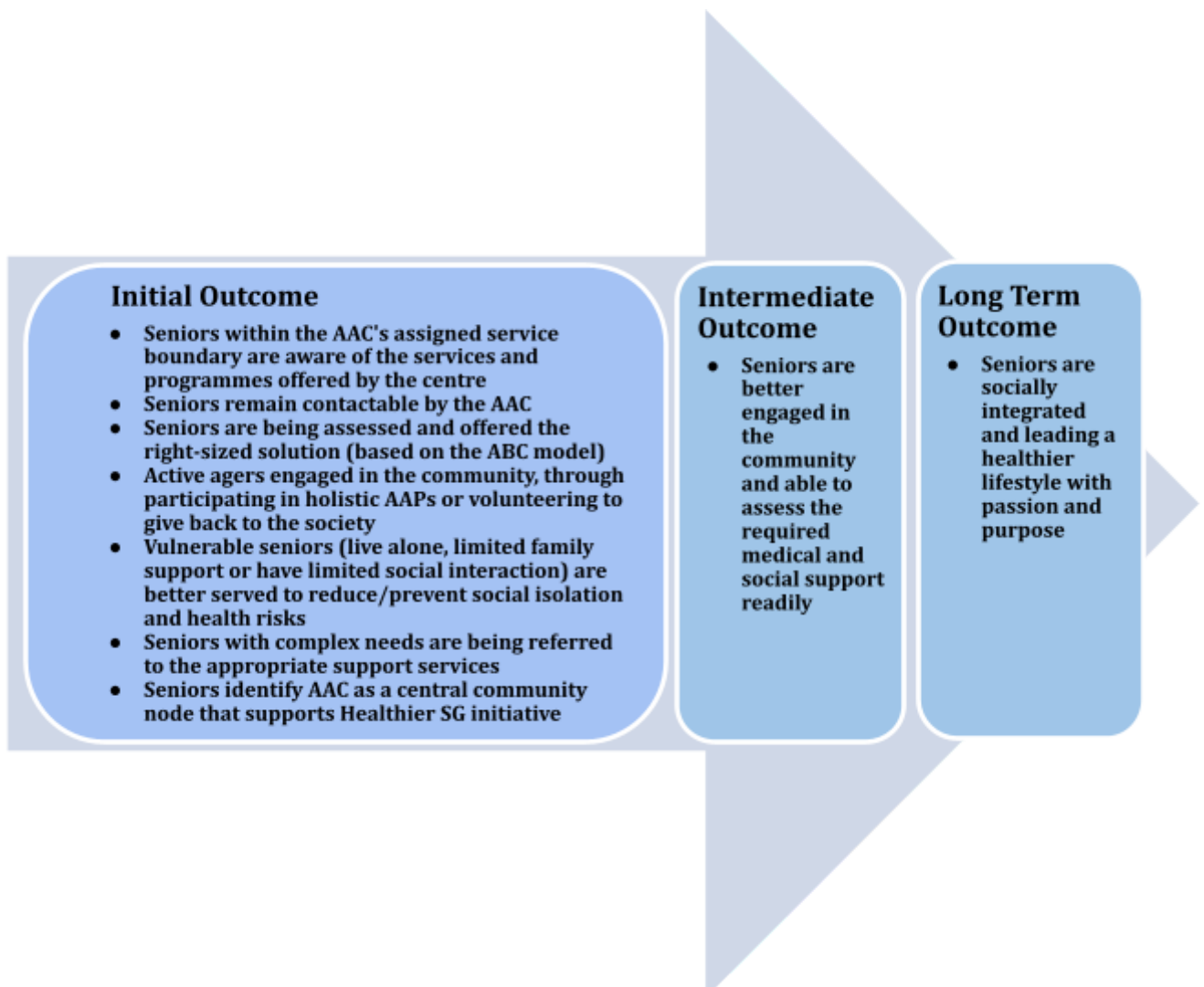
2. GOAL AND DESIRED OUTCOMES OF AACs

2.1 The goal of AACs is aligned to NTUC Health's goal, which is "Be Ageless; Enabling the ageless senior to live life with passion and purpose."

2.2 We encourage our seniors to Be Ageless by adopting NTUC Health's Care Philosophy:

- Providing our seniors with a choice
- Encouraging our seniors to be independent
- Ensuring social engagement
- Providing right-sized care solutions

2.3 We aim to achieve the following desired outcomes for the seniors we serve in stages:



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3. CONTROL OF DOCUMENTS AND RECORDS

- 3.1 Documentation refers to plans, policies, standards, procedures, forms, templates, checklists and guidelines for the management and operations of the AACs.
- 3.2 Records are any information that is used to document the progress or results of an activity or service. This can include data, logs, reports, and other documentation that captures the output of an activity or service.
- 3.3 Unless there are centre-specific requirements, all AACs should use the same set of forms, templates and checklists. CICs will need to seek approval from their Cluster Manager and Quality Management for any deviation from the standard set of documentation.
- 3.4 CICs are responsible for the update, maintenance, storage, disclosure and destruction of center-level records. Such records include but not limited to the following:
- AIC Progress Reports
 - Client Records *including registration forms, assessment records using community screening tool (CST) and client information from funders*
 - Service Records *including touchpoints with seniors (face-to-face or virtual), Active Ageing Programme listing and participation, Befriending & Buddying services, Information & Referral services, 2Ss and Service Feedback*
 - Client Service Referral Records
 - Outreach Status Records
 - Client Satisfaction Survey Records
 - Volunteer Records
 - Environmental Sanitation Programme Records
 - Incident Records
 - Complaint Records
 - Staff Training Records
- 3.5 Head, AAC and/or Cluster Managers are responsible for the maintenance, storage, disclosure and destruction of AAC-level records. CICs are responsible for the prompt provision of accurate and complete data as requested by Head, AAC and/or Cluster Managers. Such reports include but not limited to the following:
- Balance Scorecard (BSC)
- 3.6 Quality Management is responsible for the maintenance of a Master Document List that tracks the current versions and locations of all AAC-related documents.

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- 3.7 Where Google Drive is used to store electronic documentation and records, only corporate email accounts @ntuchealth should be used. Storing documents and records locally on office-issued devices should be on a need-to-basis.

4. DATA PROTECTION REQUIREMENT

- 4.1 Collection, use, disclosure and care of individuals' personal data by AACs is governed by NTUC Health's Personal Data Protection Act ("PDPA") Policy, which applies to all staff of AACs, its staff contractors, other third parties, subsidiaries, affiliates of each business unit, who generate, acquire, use, transfer, store and manage personal data and information assets.
- 4.2 'Personal data' includes NRIC number, name, address, contact information (e.g. telephone number and/or email address), photograph, video image and any other information that may identify or is personal to the client or where applicable, the Caregiver.
- 4.3 AACs can collect, use and disclose personal data for:
- a. evaluating the client's suitability and eligibility for the Programme and other programmes offered by NTUC's associates;
 - b. referring the client to other suitable programmes offered by NTUC's associates;
 - c. the performance of the Services and delivery of the Programme;
 - d. conducting the annual Community Screening and any other assessments as may be required from time to time in connection with the Programme;
 - e. the purposes described in NTUC's Privacy Policy.
- 4.4 AACs are required to exercise due care on confidential and personal data to protect against accidental or unauthorised access, use and disclosure, which include:
- a. Access to files on google drives is restricted on a need-to basis;
 - b. Do not share files with personal data to external parties unless required. Where sharing is required, encrypt the data before sending through email and share the password via a different channel (e.g. file via email, password via SMS).
Refer to "Step-by-step guide to file encryption" on how to encrypt email attachments (available on The Hive);
 - c. Do not send work emails and documents with confidential information, or files containing personal data of client, volunteer and staff to personal or unauthorised email accounts or personal devices;
 - d. Only use @ntuchealth email accounts on office-issued devices – laptops, mobile phones and tablets;

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- e. Keep personal data in any forms (e.g. hardcopies, hard disk, storage media) in secured areas that are accessible only by authorised individuals;
- f. Do not keep scanned copies of client/volunteer NRICs, whether hard copy or electronic. Transient storage, if required, for example when helping a client with application or referral, is allowed but all copies must be destroyed and disposed of once they are no longer necessary;
- g. Ensure visitor logs containing personal data (identifiable information in addition to names) are not visible to other clients and visitors, and prevent anyone from capturing this data;
- h. Ensure privacy is maintained when discussing client cases or validating client identity.

4.5 AACs have to comply with the data retention and data destruction requirements spelt out by the funders and in NTUC Health's Data Retention Policy, whichever is more stringent. In particular,

- a. The permissible methods of destruction or disposal of hardcopy and digital data;
- b. The minimum retention limitation periods^{1, 2} for the following types of data:
 - Client's records (with personal data)
 - Client's service records (with personal data)
 - Next-of-kin records
 - Volunteer records (individual and corporate)
 - Visitors records
 - CCTV records
 - Audit reports
 - Business correspondence, including correspondence with vendors or suppliers of goods and services

4.6 Any PDPA matters under AACs, such as data protection-related queries, complaints, data breaches (confirmed, potential or suspected) and disclosure of personal data as required by law, should be directed to the appointed business representative for AACs³.

¹ A "Retention Limitation Period" is defined (in NTUC Health's Data Retention policy) as the time the data and/or information is inactive to the time it is intended for destruction. In particular, the retention period for the information is measured from the start of the next financial year (i.e, 01 January). Per the NTUC Health's Privacy Policy that is made public, data retention period is typically from 6 years upwards, depending on the limitation period.

² AIC's requirement is that documentation and records should be retained for a period of at least five (5) years after the closure of MOH/AIC's financial year (i.e. 31 March) in which the documentation or record was made.

³ Quality Manager, AAC is the appointed business rep for AACs at the time of this writing.

5. SERVICE QUALITY

- 5.1 As per funder's expectation, AACs are to serve as a key point of contact for social-health matters for clients under their care, as well as for their caregivers. The scope of AAC services includes:

AAC Service / Programme	Remarks
Outreach and Community Collaboration	These services, in totality, are referred to as "ABC" .
A ctive Ageing (AAP)	
B efriending and Buddying (BnB)	
Information and Referral (I&R)	
Social Connector for Social and Lifestyle interventions	These services are collectively referred to as "2Ss" .
Community Screening	
Partnerships with Clusters /primary care physicians (e.g. family doctors) to promote overall health in the community	<i>The term "Clusters" here refers to the respective Regional Health Systems (RHS) in Singapore (i.e. National University Health Systems, SingHealth, National Healthcare Group).</i>

Reference: "Service Requirements for Active Ageing Centre Service" (as at March 2023)

- 5.2 AACs should read and understand the scope of each service detailed in the "Service Requirements for Active Ageing Centre Service" by AIC (dated as at March 2023), under Section 3 - Service of the document.
- 5.3 AACs shall provide the AAC Service to Singapore Citizens and Permanent Residents aged 60 years and above who live within the assigned service boundary. In addition, AACs shall provide the 2Ss to all Singapore Citizens and Permanent Residents aged 60 years and above, regardless of their assigned service boundary.
- 5.4 AACs should not turn away any seniors who do not meet the criteria stated in 5.3. AACs shall provide assistance to individuals where required, including referring them to other AACs, other appropriate organisations, or to AIC for follow-up.

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- 5.5 Below are approaches that centre staff should take in responding to seniors who do not meet the funding eligibility criteria:

Scenario	Centre's Approach
Senior is interested to join the centre's active ageing programmes but does not meet one or more of the following requirements: • Singaporean / Permanent Resident • Age • Service boundary	Provide the service if the centre has the resources to do so. Take the opportunity to build relations with the senior such as tapping on him/her as a volunteer.
Senior seems to have no or limited social network but does not meet one or more of the following requirements: • Singaporean / Permanent Resident • Age • Service boundary	Speak to the senior to ascertain needs and interest for befriending and/or buddying services. Put up referral via AIC's Integrated Referral Management System (IRMS) to another service provider after obtaining consent from the senior.
Senior approached centre for either information or referrals for other services, or 2Ss services	Render I&R/2Ss service to the senior, regardless of whether the senior meets the funding eligibility criteria or not.

- 5.6 AAC staff should reference the NTUC Health's Service Standards Manual in the provision of care services, particularly on the guidelines on grooming standards and effective communication.
- 5.7 Care should be delivered in a respectful manner, in safe settings that are free from abuse and where clients' personal data, including family, health, social and financial data, are safeguarded at all times. Examples of appropriate care include:
- Personal data are not discussed publicly and are shared strictly only on a need-to-basis;
 - Clients are serviced regardless of race, language, religion or economic status;
 - An environment that is open to compliments, feedback and complaints as a means of improving;
 - Complaints are handled fairly and sensitively, giving respect to the value and beliefs of the clients and caregivers;

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- A safe environment that prevent falls and injuries; any fall or injury incident are assessed, reported, followed up and monitored promptly according to the incident management procedure;
- Good hygiene practice to protect staff, clients and volunteers against adverse outcomes from infection and adhere to prevailing infection prevention and control requirements issued by the Government;
- A well-established and tested fire evacuation plan to manage fire emergency situations.

5.8 AACs shall be secular and politically neutral in their service delivery, and proselytising (including religious or political beliefs) by any person within the centre and/or to their clients is strictly prohibited.

5.9 Feedback from clients relating to services, including complaints, must be handled in an efficient and professional manner.

- a. Any negative complaint of a staff member or volunteer should be managed by the CIC, or escalated to the Cluster Manager if the affected staff is the CIC.
- b. Complaints and compliments should be recorded using the AAC Service Feedback Form, which should be reviewed by the Cluster Managers and Quality Management on a periodic basis.
- c. Complaints are considered as “incidents” and should be handled per the section on “Incident Management”.

Examples of complaints can be

- Breach of law (e.g. criminal law and Data Protection Act)
- Monetary dispute
- Physical assault/threat of physical harm
- Psychological/verbal abuse such as screams or threats (by staff, volunteers and/or clients)
- Rude/unhelpful/inappropriate behaviour (by staff, volunteers and/or clients)
- Providing incorrect/inconsistent/misleading information
- Bias or discrimination (age, race, religion, language, nationality)
- Intimidation
- Harassment
- Delay in taking action without good reason
- Failing to explain processes or advice properly
- Service fall short of client’s expectations

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- 5.10 AACs should pro-actively conduct evaluation of client satisfaction on the services rendered by the centre.
- a. Client satisfaction surveys to be conducted as per funders' requirements.
 - b. In addition to using formal survey forms, centres should also gather feedback through observations and informal interactions with the clients.
 - c. CIC to follow-up on evaluation areas that are "less than satisfactory" and determine areas for improvement as necessary.
- 5.11 Cluster Managers and CICs should recognize and provide compliments to their staff and volunteers for a job well done or for going the extra mile.
- 5.12 All staff involved in job activities that require direct interaction and/or contact with a client should be adequately trained and equipped with the knowledge and skills to deliver AAC Services.
- a. CICs are responsible to manage their own and staff's training to ensure everyone is equipped with the knowledge and skills to deliver AAC Services.
 - b. This includes attending mandatory training programmes according to their job role (refer to Appendix D).
 - c. All training attended should be documented.

6. CENTRE NAMING CONVENTION

- 6.1 The following naming conventions for the 12 AACs under NTUC Health should be used in all internal and external documentations, artefacts (posters, banners, videos etc), communications and systems:

Full Name	Short Name	Long Name
Active Ageing Centre (Care) (Bukit Batok West) / AACC (Bukit Batok West)	BBW	AACC-BBW
Active Ageing Centre (Care) (Jurong Central Plaza) / AACC (Jurong Central Plaza)	JCP	AACC-JCP
Active Ageing Centre (Care) (Kampung Admiralty) / AACC (Kampung Admiralty)	KA	AACC-KA
Active Ageing Centre (Bukit Merah) / AAC (Bukit Merah)	BM	AAC-BM
Active Ageing Centre (Bukit Merah View) / AAC (Bukit Merah View)	BMV	AAC-BMV
Active Ageing Centre (Lengkok Bahru) / AAC (Lengkok Bahru)	LB	AAC-LB
Active Ageing Centre (Marsiling) / AAC (Marsiling)	MA	AAC-MA
Active Ageing Centre (Mount Faber) / AAC (Mount Faber)	MF	AAC-MF
Active Ageing Centre (Redhill) / AAC (Redhill)	RH	AAC-RH
Active Ageing Centre (Taman Jurong) / AAC (Taman Jurong)	TJ	AAC-TJ
Active Ageing Centre (Telok Blangah) / AAC (Telok Blangah)	TB	AAC-TB
Active Ageing Centre (Whampoa) / AAC (Whampoa)	WH	AAC-WH

- 6.2 Systems mentioned above include funders' systems (e.g. AcuteNet, IRMS) and internal systems such as client relationship management and volunteer management systems.
- For external systems, AACs can add the prefix "NTUC Health" to the centre's name for identification purposes, e.g "NTUC Health Active Ageing Centre (Redhill)" or "NTUC Health AAC (Redhill)".

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- 6.3 While both short and long names can be used, depending on factors such as ease of reference and character length limitation, the same type (i.e. either short or long) should be consistently used on the same document, system or thread of communication. Such consistency will facilitate ease of searching, sorting and filtering when needed.
- 6.4 If an AAC needs to use a different naming convention, the centre's CIC should seek approval from the Cluster Manager.

7. INTERNAL & EXTERNAL COMMUNICATIONS

- 7.1 Cluster Manager has to be kept in the loop of all written communications with funding organisations and agencies. If in doubt, AAC staff should clear with the Cluster Manager before communicating.
- This excludes the routine and normal use of funders' systems such as AcuteNet and IRMS.
- 7.2 All report submissions to the funding organisation have to be pre-cleared by the Cluster Manager.
- 7.3 All external audit communications have to be pre-cleared by the Cluster Manager. Such communications include:
- Audit plan and scope of work
 - Request for information and supporting documentations
 - Audit observations – acceptance of audit observations and improvement plans have to be cleared by the Cluster Manager
- 7.4 Only staff who have been authorised by Head, AAC, can sign formal documents, such as contracts and audit reports, on behalf of the organisation.
- 7.5 Sharing of personal data with any parties has to be in accordance with the section on "Data Protection Requirements".
- 7.6 Centre staff should adopt the following guidelines when communicating with clients and/or volunteers
- a. "House Style Guidelines for staff use when drafting official letters, emails and other documents"
 - b. "WhatsApp Business Profile Handbook (Active Ageing Services)"

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- 7.7 Refer to the sections on “Service Quality” and “Incident Management” for the handling of client complaints.

8. CLIENT MANAGEMENT

8.1 Client Registration

- 8.1.1 Client registration is done using the NTUC Health AAC Registration Form.

- 8.1.2 AACs are required to obtain signed consent from seniors who want to receive the services from the centre via the Registration Form, regardless of whether they are living within or outside of the centre’s service boundary.

- 8.1.3 The consent covers the collection, use and disclosure of senior’s personal data for sharing within NTUC Health, with funding agencies (e.g. MOH/AIC), Regional Health Partners (RHSs), and other organisations for:

- a. Senior to receive services
- b. Referral to other care services if required
- c. Evaluation and research purpose.

- 8.1.4 Clients must be allowed to make the following requests regarding their personal data:

- a. Withdrawal of consent: *Client can withdraw consent for AACs to use his/her personal data*
- b. Correction: *Client may request that any incomplete or inaccurate data that AAC holds or controls be corrected*
- c. Access: *Client may ask if AAC holds or controls his/her personal data and can request access or a copy of such data*

ACC is to direct all such requests to NTUC Health’s Data Protection Officer via DPO@ntuchealth.sg, who will then work with the centre staff to handle the request on a case-by-case basis. All such requests should be documented for future reference by the organisation and in the event of dispute.

- 8.1.5 Registration of seniors should only be done by AAC staff who have been trained to explain the key terms and conditions, consent purpose and clients’ rights to their personal data.

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- 8.1.6 During the registration, staff are required to sight the physical Identity Card of the senior in the recording of the senior's name, NRIC, address and date of birth. Verbal communication of these data to the AAC is not accepted.
- 8.1.7 AAC staff must create a new client profile in the cluster's Client Management System (B3), within one week from the completion of the registration.
- 8.1.8 All client records, hard copy or electronic versions, must be securely kept and accessible only to authorised personnel on a strictly need-to basis.

8.2 Community Screening

- 8.2.1 Community Screening (CS) is the assessment tool by AIC to identify persons with health and social needs so that they can receive further assessment and intervention that aim to maximise their current function, improve their quality of life so as to live well and safely in the community.
- 8.2.2 CS is to be performed on the clients who live within the centre's service boundary:
- Upon new enrolment, and
 - To be repeated annually within the financial year.
- 8.2.3 CS should be re-administered earlier if the AAC is aware of changes in the health or social condition of the senior that may affect his/her needs, e.g. when the senior has recently been discharged from hospital; or when the senior has been facing a stressful life event, such as death of a loved one, major illness or injury.
- 8.2.4 Only certified CS-assessors can administer the community screening and it should be done face-to-face.
- Before administering the CS, the staff conducting the assessment needs to get verbal consent from the client every time. If the senior (or his/her family member) does not give consent, AACs are not allowed to proceed with the screening.
 - CS may require more than one sittings to be completed. In such a case, the "Date Assessment Signed as Complete" should be the final date that the CS was completed.
 - There is no requirement to maintain hard copy CS forms.
- 8.2.5 Seniors who have completed CS should have also signed the Registration Form.
- However, staff must not force seniors to sign the Registration Form if attempts have been made to explain the benefits and terms and conditions to the senior.

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- 8.2.6 AACs should encourage seniors to participate in Active Ageing Programmes to stay active, healthy and socially engaged. This could be done during the assessment or at subsequent interaction opportunities with the senior.
- 8.2.7 If the CS outcome identifies that the client may require Befriending/Buddying service or is to be recommended for further assessment, AAC team should follow up with the client according to the Community Screener Action Plan in Appendix A.
- a. Records of all referrals (referral systems, forms and/or email correspondences) must be kept, either in electronic or hardcopy form. If electronic records are available, AACs do not need to keep equivalent hardcopy versions.

8.3 Client Touchpoints

- 8.3.1 Centres will need to have at least one other *face-to-face interaction*, also termed as “Touchpoint”, with the in-service boundary registered clients within every financial year to ensure they are well.
- The interaction can be before or after the conduct of the Community Screening (interaction with the senior for the conduct of CS is not counted)
 - The following interactions with clients is considered as “touchpoint”:
 - Client joins an AAP;
 - Client is visited by Befrienders;
 - Centre provides face-to-face I&R service to the client;
 - Client drops by the centre, e.g. to read newspaper, enquire on certain issues, or to simply chat with a staff;
 - AAC staff meets the client in the neighbourhood or at another community event.
 - ACCs should maintain proper records of all touchpoints with clients.

9. AAC – OUTREACH & COMMUNITY COLLABORATION

- 9.1 To facilitate outreach effort, centres could be receiving from AIC dataset(s) containing data of seniors who live within the centre’s assigned service boundary. The dataset(s) to receive will depend on the centre’s request to AIC. The centre is required to comply with AIC’s requirements on usage of the data.

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	Dataset 1	Dataset 2
Client data	Name and address of seniors who have been engaged by AIC and provided consent.	Community Screener Tool (CS) responses of seniors, who have been engaged through AIC's Silver Generation Office's (SGO) Preventive Health Visits (PHV) and provided consent. Information will include name, address, NRIC of seniors.
Requirements on Using the Data	Centre staff should <u>not</u> address the senior by his/her name upon first contact with the individual. Centre <u>must not</u> state that AIC/MOH has provided the data to the centre.	Centre staff <u>can address the senior by his/her name</u> upon first contact with the individual, but make reference to the visit done by SGO earlier.
Suggested Opening Phrase	<i>"Hi, we are from (centre name). We have a centre nearby, and are offering activities and services for seniors. Can we share more with you? How can we address you?"</i>	<i>"Hi Mr/Ms/Mdm (surname of senior), we are from (centre name). Understand that Silver Generation Ambassadors visited you last (include month of SGO's visit). As it has been a few months, how have you been doing?"</i>
Other Guidelines	-	Centres can count these seniors towards centres' engagement workload if centres: i) Review clients' needs face-to-face to verify that the data is accurate as at the time of AAC/AACH's screening, before submitting the information in AcuteNet; and ii) Meet the requirements for EC engagement (i.e. completed AAC/AACH registration form and at least one other face-to-face contact with the senior).
	File must be protected and accessible only to authorised personnel strictly on a need-to basis.	File must be protected and accessible only to authorised personnel strictly on a need-to basis.

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- 9.2 Other than door-to-door, the centre may raise awareness of centre's programmes and services to the seniors and caregivers in the community by:
- Running roadshows;
 - Place AAP publicity posters at strategic locations, such as lift landings/pavilions that are frequented by seniors and caregivers, or at community nodes (e.g. Town Councils, RCs, CCs, Polyclinics);
 - Share centre's collaterals, e.g. AAP flyer, calendar or publicity posters, with local Silver Generation Office team;
 - Collaborate with other community partners (e.g. MP, PA, CC, RC and RN) to conduct programmes;
 - Partner and tap on community events (e.g. roadshows organised by grassroots)
 - Leverage on digital platforms to reach out to digital-savvy seniors and caregivers.
- 9.3 AACs are to update the Outreach Master List to track the outreach status of every senior within its service boundary. This list could also be used by the centre to plan their outreach activities, by having a consolidated oversight of:
- Seniors who had indicated interest in the AAC programmes/services and prioritise by time since last visit;
 - Seniors who had not been contactable at past visit(s) and prioritise by time since last visit;
 - Seniors who had showed no interest and allow a reasonable time gap before the next visit;
 - Seniors who had registered with the centre.
- 9.4 AACs should refer to the AAC Playbook for Outreach for practical guidance on how to conduct outreach.
- 9.5 After successfully outreaching to clients, AACs should continue to sustain and deepen the relationship, and attempt to convert these clients to "Successfully Engaged" clients.

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10. AAC – ACTIVE AGEING PROGRAMMES

10.1 Planning and implementation of AAP should be centred around the following principles:

- Empower seniors to take charge of their physical, mental and social well-being;
- Enable seniors to continue to contribute their knowledge and expertise and remain resilient, through learning and volunteerism.

10.2 There are six Active Ageing Program Pillars, termed as the CASHEW Model, that AACs should adopt. The model categorises the types of programmes and defines the target number of programmes per pillar per month.

- Be Connected - *minimum 4 per month per centre*
- Be Active - *minimum 12 per month per centre*
- Be Sharp - *minimum 8 per month per centre*
- Be Happy - *minimum 4 per month per centre*
- Be Enriched - *minimum 4 per month per centre*
- Be Well - *minimum 8 per month per centre*

10.3 The following are some approaches that centre staff could adopt in the planning and implementation of AAPs:

- a. Partner with community partners (e.g. MP, PA, CC, RC and RN);
- b. Reference resources from AIC (e.g. Wellness Wednesday)
- c. Encourage seniors to start and lead interest groups;
- d. Train-the-trainer to enable seniors to lead programmes;
- e. Conduct needs and interest analysis to understand the profile of both active and inactive seniors;
- f. Facilitate and develop rapport between seniors and trainers, especially during the first few sessions;
- g. Monitor the take-up rate of AAPs and proactively check-in with seniors who stop participation or are irregular in their participation;
- h. Have regular check-ins with trainers and seniors to improve and/or maintain the quality of programmes.

10.4 AACs are to record AAP attendance into the Online Program Registration & Attendance Taking system.

11. AAC – BERIENDING & BUDDYING

11.1 The objectives of Befriending and Buddying services and the tasks which Befrienders are required to perform are listed in the table below:

BnB Objectives	Tasks which Befrienders are required to perform
Identify signs and symptoms of overall well-being	a) Identify signs of self-neglect, or risk of self-harm; b) Report changes or red flags, such as deterioration in client's physical/mental health to the AAC staff if observed; and c) Look out for home hazards (e.g. fall risks).
Provide Emotional Support	a) Befriend and engage clients and their caregivers, and develop supportive relationships; b) Engage clients in mind-stimulating activities such as through games, learning how to use a smart phone or promoting healthy living; and c) Strongly encourage, and accompany (where possible), clients to attend at least one (1) social or recreational activities organised by the centre per month, where appropriate.

11.2 Eligibility criteria to be considered for Befriending/Budding service are:

- Senior is Singaporean or PR, and is 60 years old and above;
- Senior resides within the AAC's service boundary; and
- Senior has signed the AAC Registration Form

11.3 AACs are to refer eligible clients for Befriending or Buddying service if the Community Screener (CS) outcome indicates that the client is recommended for this service.

- Raise referral via AIC's Integrated Referral Management System (IRMS);
- Match Befrienders to provide either Befriending or Buddying service as per service requirements (refer to Appendix B⁴).

11.4 For external referrals received via the IRMS, AACs are to contact senior within 3 working days upon date of referral and schedule first appointment:

- Exceptions are allowed in the event that senior is not contactable (e.g. hospitalised, institutionalised, overseas, or not responding to phone calls and not at home);

⁴ Centre staff should be aware of the differences in service requirement between Befriending and Buddying.

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- b. Document all necessary attempts to contact the senior and correspondences that demonstrates reasonable due diligence has been exercised reasonable to contact the client;
- c. Assess suitability of befriending service, including administering CS to determine if the senior requires Befriending or Buddying service, within 10 working days after first contact;
- d. Update/Reject admission in IRMS based on the CS outcome within 3 working days after receiving consent from the client or his or her caregiver.

11.5 AACs may also receive referrals from other healthcare and community partners for Befriending and Buddying outside of IRMS.

- a. AACs should administer the CS for such referrals to ascertain that the client meets the eligibility criteria to receive Befriending/Buddying.
- b. Raise referral via AIC's Integrated Referral Management System (IRMS) if Befriending/Buddying service is recommended (based on CS outcome).

11.6 CICs or designates in charge of the Befriending/Buddying service are required to onboard new Befrienders per the volunteer onboarding process (as defined by Volunteer & Partnership). In addition, new Befrienders should undergo an orientation briefing by the AAC before they start providing the service. The briefing should cover the following areas:

- NTUC Health Purpose, Values and Services
- Volunteer Philosophy
- Relevant Sections of this Standard: Data Protection Requirements and Service Quality
- Conflict of Interest
- Code of Conduct
- Recording of visits
- Situations/Scenarios to highlight to centre staff for attention and/or follow-up
- Befrienders must maintain the client's confidentiality at all times
- No proselytising (including religious or political beliefs) to Befriendees
- Specific Instructions:
 - Befrienders should not make independent decisions regarding the assistance or follow-ups needed by the client, and shall escalate any concern, uncertainty and anomaly to the CIC or designate;
 - Befrienders to highlight to centre staff immediately if the client is suspected to be at risk of harm self or others.

11.7 AACs should assess the suitability of befriending service for the clients before matching to a Befriender. Clients who may not be suitable for this service include, but not limited to, those who:

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- Exhibit violent/unmanageable behaviours;
- Have mid- or late-stage dementia;
- Are physically too weak to be engaged.

11.8 For every new Befriender-Befriendee match, CICs or designates are to conduct home visits along with the assigned Befriender(s) at the first two visits.

- The objective of the visit is to understand the profiles and needs of the Befriendee;
- After the joint visit, CIC or designate should discuss with the Befriender(s) on the befriending/buddying care plan objectives and implementation.

11.9 CICs are required to conduct surprise home visits of Befriendees, without the presence of Befrienders, at least once every quarter to:

- Ensure safety and well-being of both Befriendees and Befrienders
- Build relationship with the Befriendees and encourage senior to join centre's programmes;
- Monitor clients' conditions (assess the need to increase or reduce service level⁵);
- Obtain feedback on the Befriending or Buddying service; and
- Identify any unmet needs or inappropriate conduct.

11.10 CICs are required to review the home visit records to:

- Ensure accuracy and completeness;
- Identify and follow-up on issues or red flags that required intervention;
- Ensure Befriending or Buddying service requirements as stipulated by AIC are met

11.11 Client should be discharged from the Befriending/Buddying service when one of the following situations happen:

Reason for discharge	Action by AAC
Client has passed on	Update BnB Client Masterlist with the Date of Decease
Community re-screening indicates that client no longer needs the service	Client to sign Discharge Form, with reason for discharge as "No BnB required".
Client or his/her caregiver (next of kin) informs the centre that senior no longer needs or wants the service	Client to sign Discharge Form, with details of the declination - who spoke to who, date, and reason(s) for declination.

⁵ The desired goal of befriending is that there is improvement in the overall wellbeing of clients such that the service level can be reduced (i.e. from Befriending to Buddying or from Buddying to Active Senior).

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12. AAC – INFORMATION & CARE REFERRAL

- 12.1 AACs are expected to be the key point-of-contact for social health matters to seniors and their caregivers by providing the following services upon request:
- a. Information on relevant schemes, grants and support available for seniors or their caregivers;
 - b. Refer seniors to other agencies or services so that seniors and caregivers can receive appropriate and timely support;
 - c. Escalate to AIC if seniors have further needs beyond the AAC capability or knowledge;
 - d. Provide support to seniors with data literacy issues, such as helping with letter reading or digital access; and
 - e. Support seniors in enrolment and/or assist with queries related to Healthier SG.
- 12.2 Information & Care Referral services have to be provided to all seniors who approached the centre, regardless of whether they live in the assigned service boundary.
- 12.3 AACs are to record all I&R services rendered in the “AAC IR Client Service Record Form” and ensure the record is up-to-date on a monthly basis.
- a. The actions taken and the outcome of the referral (pending, successful or unsuccessful) should be tracked.
 - b. It is recommended that CICs review the service records on a monthly basis.

13. AAC – 2Ss

- 13.1 Under Healthier SG, healthcare professionals may refer seniors to AACs for the purpose of adherence to their health plan. AACs are expected to contact and follow up with seniors as referred by the healthcare professionals.
- 13.2 The scope of 2Ss include:
- a. Connecting seniors to Active Ageing Programmes in adherence to their Health Plan (i.e. Social Connector for Social and Lifestyle Interventions)
 - b. Facilitating seniors who require Measurement and Monitoring in their vital signs (i.e. Community Screening)

For more details, AACs should refer to the “Service Requirements for Active Ageing Centre Service” by AIC (dated as at March 2023) and all materials shared by the funder at engagement sessions.

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- 13.3 AACs are to provide 2Ss to seniors who walk-in to the AAC or seniors who are referred via Primary Care Networks (PCN) and/or Community Teams, regardless of whether the senior stays within the AAC's service boundary or not.
- 13.4 AACs are expected to adhere to the documentation requirements and the indicated recommendations and frequency in relation to the management of referrals, as defined by AIC.
- 13.5 AACs must maintain proper documentation for tracking, reporting, and audit purposes.

14. AAC – ADMIN FUNCTIONS

- 14.1 The following administrative functions are not included in the current version of this document and AACs are to refer to documentation and guidance from the respective process owner:

Administrative Tasks	Process Owner (Department)
Management of micro-jobbers supporting the AAC functions	Human Resource
Onboarding and management of volunteers supporting the AAC functions	Volunteer & Partnership
Procurement of goods and services	Procurement
Maintenance of centre facilities and cleanliness	Facilities Management
Online system: Data entry, export and dashboard • Bumblebee (B3) system • Online Program Registration & Attendance Taking (OPR) system	AAC, supported by Digital & Technology

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15. MEETINGS & REPORTING REQUIREMENTS

15.1 Internal Meetings & Reporting

15.1.1 Regular internal meetings should be convened with stakeholder groups to discuss and make decisions on AAC management and operations issues and opportunities.

Internal Meetings	Chairperson/Attendees	Frequency
AAC Management Meeting	Chairperson: Head, AAC Required Attendees*: • AAC Management Team • Finance Business Rep for AAC • HR Business Rep for AAC • Corp Comm Business Rep for AAC • Strategy & Transformation Business Rep for AAC	Monthly
Cluster-Level Meeting	Chairperson: Cluster Manager Required Attendees*: • Cluster Manager, AAC • Head, AAC • Quality Manager, AAC • CICs of the Cluster	Monthly
Ops Meeting	Chairperson: Cluster Manager (<i>rotational basis</i>) Required Attendees*: • AAC Management Team • All CICs (AAC, SDC-TG, SAC) • BU Reps (by invitation)	Monthly

** Other personnel may be invited to attend the meeting as and when necessary.*

15.1.2 AAC Management Team comprises of the following staff/representatives:

- Head, AAC
- Cluster In-Charge
- Volunteer Management & Partnership
- Befriending Management
- Eco System Partnership / Program
- Innovation Project
- Strategic Ops.
- Quality Management
- Community of Care (2.0)

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15.1.3 Chairperson of the meeting should ensure that meetings are minuted and that action items arising from the meetings are assigned and tracked to closure.

15.1.4 CICs should ensure internal management reports are completed/submitted by the stipulated timeline. The required internal reports include:

- Balanced Scorecard (BSC) - report due 5th of every month

15.2 External Reporting

15.2.1 The table below summarises the report(s) that AACs are required to submit to external parties:

Report	External Party	Frequency	Data Provider	Reviewer	Approver	Submitter
Progress Report	AIC	Half-yearly	(Who to provide the required data?) CIC	(Who to review before submission?) Cluster Manager	(who to give approval to submit?) CEO (or designate)	(who can submit the report?) Personnel authorised by Approver

15.2.2 CICs are required to maintain records and documentation of the services rendered, and interactions with clients to substantiate their reporting of data submitted to AIC, including but not limited to:

- Outreach efforts;
- Registration and CS records;
- Touchpoint and AAP attendance records;
- Befriending and Buddying client and interaction records.

16. INCIDENT MANAGEMENT

16.1 AACs are to adopt the overarching incident framework defined by NTUC Health's Risk & Compliance in the areas of:

- Incident Classification
- Incident Reporting
- Incident Notification

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- 16.2 In addition to complying with internal incident reporting and notification protocol, AACs are also required to comply with AIC's guidelines on Safety and Incident Management. Where there are differences, the stricter of the two requirements will apply.
- 16.3 Client complaints and incidents that involve human safety or could result in potential adverse publicity must be promptly escalated to the Cluster Manager within the first hour.
- 16.4 AACs must keep detailed records of all incidents and follow-up actions taken, using the AAC Service Feedback Form. The guidelines stated in the Form should be observed, which includes:
- Timeline for reporting staff to inform CIC, for CIC to take the first response and for Cluster Manager to review the feedback and actions taken;
 - A copy of the completed form to be sent to the Head of AAC and to Quality Management.
- 16.5 Incidents including, but not limited to, the following nature will need to be reported to AIC using AIC's Incident Reporting Template:
- Outbreak of fire on centre premises leading to serious damage to property or injuries
 - Illegal activities/ crimes committed within centre premises
 - Data breaches or malicious data-related activities
 - Unnatural deaths /Suicide or attempted suicide/ Death at clients' home, undetected
 - Terrorist activity, bomb threats and other potentially life-threatening incidents

17. INTERNAL AUDIT

- 17.1 Quality Management shall conduct internal audits at planned intervals to provide information on whether the operations of the AAC:
- a. Conforms to the requirements of the funders and of this Standard;
 - b. Is effectively implemented and maintained;
 - c. Can be improved or enhanced (in terms of data accuracy and completeness, process efficiency, service quality etc)
- 17.2 The audit plan, which includes the audit criteria and scope, will be established by Quality Management and approved by the Head, AAC.

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Appendix A - COMMUNITY SCREENER ACTION PLAN

Active Aging Programme Criteria [A]	
AUA 1 - 6	Encourage clients to participate in the centre's Active Ageing Programmes
Buddying and Befriending Criteria [B]	
Buddying	Befriending
has ONE of these FOUR criteria	has any TWO of these FOUR criteria
• E1a (code "1") + F2 (code "3" and above) – <i>Living alone or as good as living alone, and feels lonely</i> • E1b (code "1") – <i>Client is living with someone who requires high care needs from him/her</i> • E2 (code "1") – <i>Has limited or no support from anyone</i> • F3 (code "1") – <i>Has limited or no social interaction</i> * Put up referral via IRMS.	
Further Assessment Criteria [C]	
(I) Cognitive Issues • B1 – Short term memory • C2 – IADL capacity • D2 – More difficulties in completing day to day activities	Refer clients to "NTUC Health Care Coordination Team". <i>This team will refer clients to NTUC Health services, Community Case Management Service (CCMS), etc as needed.</i>
(II) Functional Issues • D1 – Fall risks • D2 – More difficulties in completing day to day activities	Refer clients to "NTUC Health Care Coordination Team".
(III) Medical Issues • D3 – Involuntary weight loss • D4 – Chronic disease not treated/monitored	Refer clients to Community Nursing / Health Posts.
(IV) Self Harm Risk • E3 – <i>Risk of harm to self or others</i> • F1b – <i>Risk of harm to self or others</i>	Refer clients to Community Case Management Service (CCMS) - NTUC Health or external provider.

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(V) Others • Advance Care Planning (ACP) • Caregiver Training/Education and Support • Case Management and Coordination • Chronic Disease Management - Health Coaching and Monitoring • Fall Prevention Screening & Education • Health & Geriatric Assessment • Medical consolidation and interim medication packing • Smoking Cessation • Social Prescribing • Weight Management • Management of geriatric syndromes and rehabilitation	Refer to Healthier SG Team for triage. • SingHealth: eReferral form
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Appendix B - BEFRIENDING/BUDDYING SERVICE REQUIREMENTS

Service Type	Befriending	Buddying
Eligibility Criteria	• Senior is Singaporean or PR, and is 60 years old and above • Senior resides within the AAC's service boundary • CS is done and outcome indicates that client is recommended for Befriending/Buddying service	
Frequency* of Contact with Client	At least once a week - minimum of 2 face-to-face sessions per month	At least once a month - minimum of 1 face-to-face session per 3 months
Mode of Service Delivery	Face-to-face Sessions / Phone or Video Calls	

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Appendix C - "CLIENT STATUS" TERMINOLOGIES

Client Status	Definition
Registered	• Senior has signed the AAC Registration Form
Successfully Engaged	• Senior is Singaporean or PR, and is 60 years old and above • Senior resides within the AAC's service boundary • Senior has signed the AAC Registration Form • Senior has a Community Screener (CS) assessment done within the past one year • Centre has at least one other touchpoint (face-to-face) with the senior in the current Financial Year
Active Befriended/Buddied Senior	• Senior is "Successfully Engaged" • Senior has consented to receive Befriending or Buddying service • Senior has been admitted for Befriending/Buddying service in IRMS • Senior has received Befriending/Buddying service from the centre in the current Financial Year • Centre meets Befriending or Buddying service requirements in the current Financial Year • Senior is not discharged in the current Financial Year
Successfully Outreached	• Senior is Singaporean or PR, and is 60 years old and above • Senior resides within the AAC's service boundary • Centre has at least 2 successful face-to-face or virtual interactions with the senior in the current Financial Year

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Appendix D - MANDATORY TRAININGS

Course Name	Description	Target Audience
Community Screener (CS) Training	Enable AAC and IBP staff to become competent CS assessors so they could help identify senior who is presently or potentially at risk of health and social vulnerabilities. This allows the centre to pick up issues proactively and carry out timely intervention for seniors.	Staff who conduct Community Screener for clients.
Community Befriending for Seniors (CBS)	Help Befrienders understand and grasp key components and mechanisms of befriending seniors and be empowered to provide effective and sound befriending services.	Staff who provide Befriending/Buddying services to clients and/or manage befrienders.
Essential Foundation in Volunteer Management (VM)	Provide an introduction to volunteer management framework, principles, and best practices. It aims to develop staff in volunteer management, including job designing, recruiting, managing, motivating, and retaining to meet organisational needs.	Staff who manage volunteers and/or involve in volunteer management.
Integrated Referral Management Systems (IRMS) e-Training (Befriending)	Complimentary e-Learning on how to use IRMS to create and/or accept referrals, including buddying or befriending services.	Staff involve in raising and receiving befriending referrals.
Community-based Vital Signs Monitoring (CBVSM)	Customised to equip learners with foundational knowledge and skills in assisting seniors with self-monitoring and self-recording of vital signs, and encouraging seniors to seek medical attention when vital signs exceed the prescribed/standard threshold accordingly	Staff who assist clients in vital signs monitoring.
Introduction to Community Connector Course (CC)	Customised to equip Active Aging Centre (AAC) staff with the foundational knowledge and skills to act as the link between primary care and community; and getting seniors to follow up on his/her Health Plan.	Staff involve in the integration of health and social care.
Community Connector Supervisor Programme (CCS)	Equips learners with knowledge and skills to monitor the performance of staff who attended the baseline programme against the programme outcomes; and make sound judgement of how well the staff apply their training.	Staff who have supervisory roles.

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Course Name	Description	Target Audience
Community Befriending of Seniors Specialists Training (CBSS)	Designed for staff of centres providing Budding and Befriending services who have been identified to train and equip their Befriending Volunteers in befriending knowledge and skills.	Staff who supervise, mentor, and/or train volunteers in befriending services.
Environmental Control Coordinator (ECC)	To understand the importance of the Environmental Sanitation Regime, the roles and responsibilities of Premises Managers and Environmental Control Coordinators (ECCs), pest management, cleaning and disinfection, and indoor air quality to enable learners to develop and monitor the implementation of an Environmental Sanitation Programme as an Environmental Control Coordinator.	Staff who will be assuming the role of an Environmental Control Coordinator (ECC) or who have interest to take up the role of ECC. <i>Each centre should have at least 2 staff who are trained in this.</i>
Standard First Aid Provider	To empower participants to effectively respond to first aid emergencies in the workplace until the arrival of medical assistance.	<i>Each centre should have at least 2 staff who are trained in this.</i>
Hands-only CPR + AED Provider	To be able to perform basic CPR and to operate the AED in an emergency situation.	<i>Each centre should have at least 2 staff who are trained in this.</i>

Note that the above list is not exhaustive and AACs should, from time to time, take guidance from the funders and AAC management on the training requirements.